INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Adjuvant abemaciclib + endocrine therapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00619
Version 4a

Continuous oral treatment

Treatment stages

01 **A 28-day reference block**
Treatment → checks → review

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	5
03	Symptoms and supportive care	9
04	Sources and further information	15

My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

For eligible high-risk node-positive early breast cancer, adding abemaciclib to endocrine therapy reduces the risk of recurrence after surgery.

CDK4/6 INHIBITOR

Abemaciclib

Slows the cell-division checkpoint

CDK4/6 checkpoint → Checkpoint inhibited → Cell cycling slows

Abemaciclib inhibits CDK4 and CDK6 proteins. These proteins help cells move towards division. It is combined with endocrine treatment to reduce recurrence risk in eligible high-risk early breast cancer.

Start the prescribed diarrhoea rescue plan at the first loose stool and contact the team. Breathlessness, a swollen painful leg, fever or jaundice need prompt assessment.

HORMONE-DIRECTED TREATMENT

Your endocrine treatment

Reduces the hormone signal

Hormone signal → Signal reduced or blocked → HR-positive growth reduced

The chosen endocrine medicine either reduces oestrogen production or blocks its action. An aromatase inhibitor may need ovarian suppression before menopause. The exact drug and duration are prescribed separately.

Tell the team about joint pain, hot flushes, vaginal symptoms and mood changes. The bone-health and clot-risk discussion depends on the drug chosen.

Your treatment aim

Who this treatment is for

HR-positive/HER2-negative node-positive early breast cancer at high recurrence risk: four or more positive axillary nodes, or one to three with tumour at least 5 cm and/or grade 3. The NCCP timing, prior-treatment and other exclusions remain applicable; this is not a node-negative or metastatic regimen.

NCCP 00619 v4a · official regimen

Schedule and duration

The usual abemaciclib starting dose is 150 mg twice daily; a lower dose or altered schedule may be prescribed. Abemaciclib is taken twice daily continuously for up to two years with endocrine therapy, unless recurrence or unacceptable toxicity requires an earlier change. A four-week dispensing block is not a rest cycle. Endocrine treatment normally continues beyond abemaciclib. At completion of the planned two-year abemaciclib course, continue the separately agreed endocrine and follow-up plan.

NCCP 00619 v4a · official regimen

What the studies tell us

Adding a planned course of abemaciclib to endocrine therapy reduced the chance of invasive cancer returning in eligible high-risk, node-positive early breast cancer. Longer follow-up also showed an overall-survival benefit. These group results do not predict your own outcome.

How to interpret this

Study results describe groups with defined treatment and eligibility. Ask about your own likely benefits, important harms, alternatives and remaining uncertainty.

monarchE primary OS and seven-year outcomes, Johnston et al; online 2025, Annals of Oncology 2026

monarchE: primary OS and seven-year outcomes

Studies describe groups. Ask how closely the treatment and study population match your situation.

Your team can discuss the strength and limitations of the evidence alongside treatment risks and alternatives.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Report new breathlessness, chest symptoms, severe diarrhoea, inability to drink, jaundice, a painful swollen leg or a severe rash promptly. You do not need to identify which medicine caused the symptom.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

Treatment and monitoring over time

Columns show sequence and general intervals, not equal periods or booked appointments. The team confirms who arranges each test, who reviews results and who authorises treatment or restart.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00619 v4a · official regimen · EMA Verzenios current product information; checked 2026-10-25

THE WHOLE TREATMENT JOURNEY

Adjuvant abemaciclib + endocrine therapy: treatment and checks

Adjuvant abemaciclib + endocrine therapy

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

HR-positive / HER2-negative early breast cancer · adjuvant

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before first dose	First two months	Next two months	Later treatment	Two-year completion / change
Abemaciclib and endocrine partner NCCP 00619 · SMPC	Confirm high-risk eligibility and treatment timing	Twice daily continuously; no rest week	Continue if tolerated	Up to two years; endocrine treatment continues	Review endocrine plan separately
full blood count (FBC) / renal / liver / bone profile NCCP 00619 · SMPC	Baseline blood tests	Every two weeks	Monthly	As clinically indicated	Follow unresolved abnormalities
electrocardiogram (ECG) / heart rhythm NCCP 00619 · SMPC	As clinically indicated	Repeat if clinically indicated	No routine Day-14 ECG schedule	Symptoms/ risk guide testing	Follow any identified problem
Diarrhoea / hydration NCCP 00619 · SMPC	Rescue medicine and contact plan ready	Start action plan at first loose stool	Continue symptom and intake review	Contact team between visits for persistent symptoms	Follow recovery; do not self-restart held doses

Care / check	Before first dose	First two months	Next two months	Later treatment	Two-year completion / change
Clots / lungs / infection NCCP 00619 · SMPC	Review prior VTE and breathing symptoms	Prompt assessment of warning symptoms	Continue symptom review	Report new leg swelling, chest or breathing symptoms	Symptoms still need assessment after stopping
Bone / endocrine / follow-up NCCP 00619 · SMPC	Assess fracture risk; confirm aromatase inhibitor (AI)/ ovarian function suppression (OFS) or other partner	Support hormone-related symptoms	Continue endocrine and bone plan	Early-breast surveillance and risk-based bone review	Endocrine treatment and surveillance continue

What each check is for

1 Before the first dose

Before first dose

Check eligibility, organ function and the exact treatment/support plan.

Ask / act: Confirm who arranges each check and who reviews it.

2 Early toxicity checks

First two months

Important toxicity can begin before the first routine review.

Ask / act: Keep early appointments and use the agreed symptom-contact plan.

3 Benefit, tolerability and adherence

Later treatment

Longer-term treatment still needs clinical review.

Ask / act: Ask about symptom support, results and the next decision.

4 Completion or treatment change

Two-year completion / change

The targeted medicine and endocrine partner may have different stop dates.

Ask / act: Confirm each medicine and the ongoing follow-up plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Checks before and during treatment

Baseline tests and ongoing tests have different purposes.

Before the first dose

FBC, renal, liver and bone profile; ECG as clinically indicated. Confirm node-positive high-risk eligibility, definitive surgery and timing: within 16 months of resection and up to 12 weeks of endocrine treatment after the last non-endocrine treatment before starting. Assess VTE history, bowel function, all medicines, endocrine partner, ovarian suppression if using an AI before menopause, and bone/reproductive health.

During treatment

FBC, renal, liver and bone profile every two weeks for the first two months, monthly for the next two months, then as clinically indicated. More frequent liver monitoring for grade 2 or worse liver abnormalities. ECG is driven by clinical need; no routine ribociclib Day-14 ECG schedule. Early and ongoing diarrhoea review, hydration/intake, infection, clots, breathlessness and adherence; prompt action between appointments. Creatinine can rise through inhibited tubular secretion without a true fall in filtration; assess the clinical context and alternative renal measures if needed.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Abemaciclib · every day	 **Days 1–28** Twice-daily tablets · no planned week off
Endocrine treatment	 **Days 1–28** Continue as prescribed
Blood counts / infection	 **Days 1–28** Tests and symptoms guide review
Energy and bowel symptoms	 **Days 1–28** Timing varies; report changes early

Treatment days: Abemaciclib is taken twice daily continuously, with endocrine treatment, for up to two years if appropriate. A 28-day block is shown for orientation; there is no planned tablet-free week. Endocrine treatment usually continues longer.

The bigger picture: Use the whole-treatment journey for surgery, radiation, scans and longer-term checks.

Read the phase labels: The prescription and team's instructions determine your actual doses and any interruptions.

> **Why these bands stay broad:** Treatment bands show the source schedule. Symptom bands show possible patterns, not measured probabilities; serious effects can occur at any time. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00619 v4a · official regimen ↗

> **There are no guaranteed "safe days".** Fever, shivers or suddenly feeling unwell require immediate contact. Serious symptoms override the timeline or diary.

Living with treatment

Supportive medicines and the endocrine partner have their own plan.

Diarrhoea rescue from the first loose stool

Have the prescribed antidiarrhoeal treatment available before starting. Begin the agreed plan at the first loose stool, increase fluids as advised and contact the team early; persistent symptoms or dehydration may require a treatment hold.

Nausea support

NCCP classifies abemaciclib as oral moderate–high emetic risk. Agree preventive or rescue antiemetics using individual risk and local oral-systemic anticancer therapy (SACT) policy.

Blood, liver and symptom checks

Attend early two-weekly then monthly checks even when tablets are taken at home. Seek prompt advice for infection, jaundice, new breathing symptoms or possible blood clots.

Endocrine and bone support

Discuss symptoms and bone protection for the actual endocrine partner. Ovarian suppression is required with an AI before menopause. Endocrine treatment normally continues after the two-year abemaciclib course.

Medicines and supply

Take tablets whole at the prescribed regular times. There is no routine week off. Check grapefruit/CYP3A interactions and do not double a missed or vomited dose.

When the plan changes

Continue, hold or stop

The team confirms the plan for each medicine and the next review. Do not restart a clinically held treatment from this educational timeline.

Practical support

Diarrhoea rescue from the first loose stool

Nausea support

Blood, liver and symptom checks

Endocrine and bone support

Medicines and supply

Taking tablets / attending treatment

Practical treatment details

Supply and appointments

Request the next supply before tablets run out.

This oral treatment does not itself require a central line or infusion pump.

Questions to ask

Which checks happen before and during treatment?

FBC, renal, liver and bone profile; ECG as clinically indicated. Confirm node-positive high-risk eligibility, definitive surgery and timing: within 16 months of resection and up to 12 weeks of endocrine treatment after the last non-endocrine treatment before starting. Assess VTE history, bowel function, all medicines, endocrine partner, ovarian suppression if using an AI before menopause, and bone/reproductive health. FBC, renal, liver and bone profile every two weeks for the first two months, monthly for the next two months, then as clinically indicated. More frequent liver monitoring for grade 2 or worse liver abnormalities. ECG is driven by clinical need; no routine ribociclib Day-14 ECG schedule. Early and ongoing diarrhoea review, hydration/intake, infection, clots, breathlessness and adherence; prompt action between appointments. Creatinine can rise through inhibited tubular secretion without a true fall in filtration; assess the clinical context and alternative renal measures if needed.

Do I need to enter dates?

No. This map shows general timing without personal information. Your hospital arranges actual appointments; ask who books each test and who reviews its result.

What if a dose is missed or treatment is held?

Take abemaciclib twice daily at regular times with or without food. Swallow whole; do not split, chew or crush. Avoid grapefruit. Do not replace a missed or vomited dose. A clinical hold and restart require the treating team’s instructions. A hold may affect one medicine rather than the whole combination; confirm each component.

What happens after treatment changes or ends?

At completion of the planned two-year abemaciclib course, continue the separately agreed endocrine and follow-up plan. Keep the separately agreed disease follow-up, endocrine and bone-health plan.

Explain the plan back: Which medicine is taken when, which test is due next, who reviews it, and which symptoms mean you should call today?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Adjuvant abemaciclib + endocrine therapy · supportive-care plan

Active medicines: Abemaciclib + Your endocrine treatment

Emetogenic risk: Oral moderate–high

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Have the prescribed antidiarrhoeal treatment available before starting. Begin the agreed plan at the first loose stool, increase fluids as advised and contact the team early; persistent symptoms or dehydration may require a treatment hold.
- NCCP classifies abemaciclib as oral moderate–high emetic risk. Agree preventive or rescue antiemetics using individual risk and local oral-systemic anticancer therapy (SACT) policy.
- Attend early two-weekly then monthly checks even when tablets are taken at home. Seek prompt advice for infection, jaundice, new breathing symptoms or possible blood clots.
- Discuss symptoms and bone protection for the actual endocrine partner. Ovarian suppression is required with an aromatase inhibitor (AI) before menopause. Endocrine treatment normally continues after the two-year abemaciclib course.
- Take tablets whole at the prescribed regular times. There is no routine week off. Check grapefruit/CYP3A interactions and do not double a missed or vomited dose.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

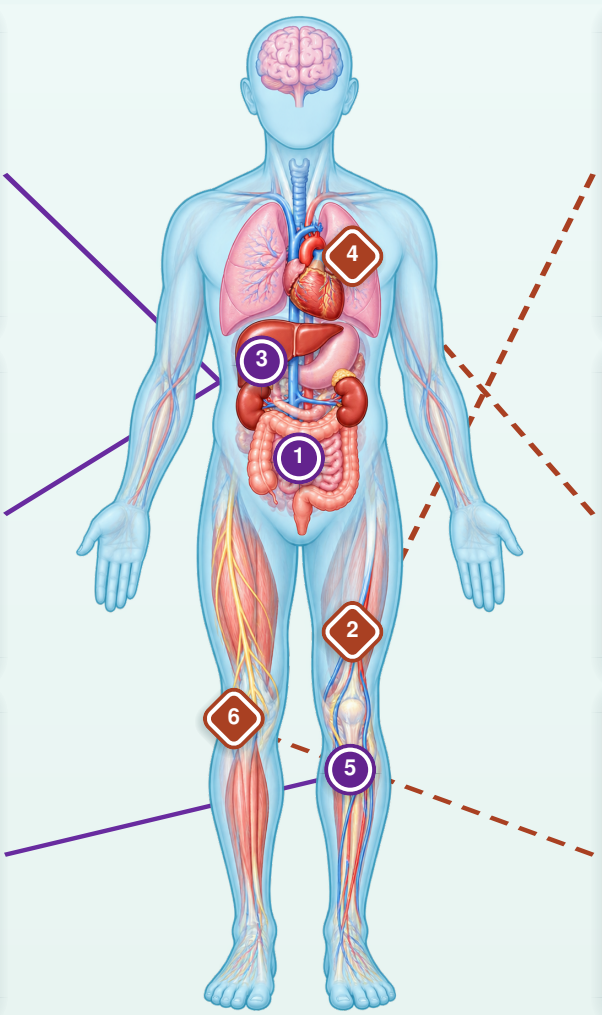
Adjuvant abemaciclib + endocrine therapy

The usual abemaciclib starting dose is 150 mg twice daily; a lower dose or altered schedule may be prescribed. Abemaciclib is taken twice daily continuously for up to two years with endocrine therapy, unless recurrence or unacceptable toxicity requires an earlier change. A four-week dispensing block is not a rest cycle. Endocrine treatment normally continues beyond abemaciclib.

· selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Diarrhoea / dehydration

Abemaciclib

Often starts early. Begin your prescribed diarrhoea plan and contact the team; persistent symptoms may require a treatment hold.

2 Low neutrophils / infection

Abemaciclib

Keep blood tests. Fever, shivers or sudden illness: contact oncology immediately.

3 Liver injury

Abemaciclib

Keep liver blood tests. Report jaundice, dark urine or upper-abdominal discomfort promptly.

4 Lung inflammation

Abemaciclib · uncommon but important

New cough or breathlessness needs prompt assessment.

5 Blood clot

Abemaciclib

A painful swollen leg or new chest / breathing symptoms needs urgent assessment. Severe breathlessness or chest pain: emergency help.

6 Joints and bone health

Endocrine partner

aromatase inhibitor (AI) and ovarian suppression can thin bones; discuss fracture risk and the monitoring plan.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report new breathlessness, chest symptoms, severe diarrhoea, inability to drink, jaundice, a painful swollen leg or a severe rash promptly. You do not need to identify which medicine caused the symptom. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00619 v4a · official regimen ↗**
   https://healthservice.hse.ie/documents/6415/619_Abemaciclib.pdf
2. **EMA Verzenios current product information; checked 2026-10-25**
   https://www.ema.europa.eu/en/documents/product-information/verzenios-epar-product-information_en.pdf
3. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
4. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
5. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
6. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
7. **NOGG · section 7, aromatase inhibitors and fracture risk**
   https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
8. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
9. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
10. **monarchE: seven-year invasive disease-free survival · primary report**
    https://www.annalsofoncology.org/article/S0923-7534(25)04948-8/fulltext
11. **monarchE primary OS and seven-year outcomes, Johnston et al; online 2025, Annals of Oncology 2026**
    https://pubmed.ncbi.nlm.nih.gov/41110697/
12. **NCCP antiemetics v8, Table 1 pp4–5**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf